

PREScribing INFORMATION

For the use of a Registered Medical Practitioner or a Hospital or a Laboratory only.

SITACORD 50 and SITACORD 100

(Sitagliptin Film-Coated Tablets 50 mg and Sitagliptin Film-Coated Tablets 100 mg)

1. BRAND OR PRODUCT NAME:

SITACORD 50 (Sitagliptin Film-Coated Tablets 50mg)

SITACORD 100 (Sitagliptin Film-Coated Tablets 100mg)

2. NAME AND STRENGTH OF ACTIVE SUBSTANCE(S):

Sitagliptin Phosphate Anhydrous equivalent to Sitagliptin; 50 mg and 100 mg

3. PRODUCT DESCRIPTION:

SITACORD 50: Light beige color, round, biconvex, film coated tablet, debossed with “II4” on one side and plain on other side.

SITACORD 100: Beige color, round, biconvex, film coated tablet, debossed with “II3” on one side and plain on other side.

4. PHARMACOLOGICAL PROPERTIES:

Pharmacotherapeutic group: Sitagliptin phosphate is an orally-active, potent, and highly selective inhibitor of the dipeptidyl peptidase 4 (DPP-4) enzyme for the treatment of type 2 diabetes. The DPP-4 inhibitors are a class of agents that act as incretin enhancers. By inhibiting the DPP-4 enzyme, sitagliptin increases the levels of two known active incretin hormones, glucagon-like peptide-1 (GLP-1) and glucose-dependent insulinotropic polypeptide (GIP). The incretins are part of an endogenous system involved in the physiologic regulation of glucose homeostasis. When blood glucose concentrations are normal or elevated, GLP-1 and GIP increase insulin synthesis and release from pancreatic beta cells. GLP-1 also lowers glucagon secretion from pancreatic alpha cells, leading to reduced hepatic glucose production. This mechanism is unlike the mechanism seen with sulfonylureas; sulfonylureas cause insulin release even when glucose levels are low, which can lead to sulfonylurea-induced hypoglycemia in patients with type 2 diabetes and in normal subjects. Sitagliptin is a potent and highly selective inhibitor of the enzyme DPP-4 and does not inhibit the closely-related enzymes DPP-8 or DPP-9 at therapeutic concentrations. Sitagliptin differs in chemical structure and pharmacological action from GLP-1 analogues, insulin, sulfonylureas or meglitinides, biguanides, peroxisome proliferator-activated receptor gamma (PPAR γ) agonists, alpha-glucosidase inhibitors, and amylin analogues

Mechanism of action

Sitagliptin is a member of a class of oral anti-hyperglycemic agents called dipeptidyl peptidase 4 (DPP-4) inhibitors, which improve glycemic control in patients with type 2 diabetes by enhancing the levels of active incretin hormones. Incretin hormones, including glucagon-like peptide-1 (GLP-1) and glucose-dependent insulinotropic polypeptide (GIP), are released by the intestine throughout the day, and levels are increased in response to a meal. The incretins are part of an endogenous system involved in the physiologic regulation of glucose homeostasis. When blood glucose concentrations are normal or elevated, GLP-1 and GIP increase insulin synthesis and release from pancreatic beta cells by intracellular signaling pathways involving cyclic AMP. Treatment with GLP-1 or with DPP-4 inhibitors in animal models of type 2 diabetes has been demonstrated to improve beta cell responsiveness to glucose and stimulate insulin biosynthesis and release. With higher insulin levels, tissue glucose uptake is enhanced. In addition, GLP-1 lowers glucagon secretion from pancreatic alpha cells. Decreased glucagon concentrations, along with higher insulin levels, lead to reduced hepatic glucose production, resulting in a decrease in blood glucose levels. The effects of GLP-1 and GIP are glucose dependent such that when blood glucose concentrations are low, stimulation of insulin release and suppression of glucagon secretion by GLP-1 are not observed. For both GLP-1 and GIP, stimulation of insulin release is enhanced as glucose rises above normal concentrations. Further, GLP-1 does not impair the normal glucagon response to hypoglycemia. The activity of GLP-1 and GIP is limited by the DPP-4 enzyme, which rapidly hydrolyzes the incretin hormones to produce inactive products. Sitagliptin prevents the hydrolysis of incretin hormones by DPP-4, thereby increasing plasma concentrations of the active forms of GLP-1 and GIP. By enhancing active incretin levels, sitagliptin increases insulin release and decreases glucagon levels in a glucose-dependent manner. In patients with type 2 diabetes with hyperglycemia, these changes in insulin and glucagon levels lead to lower hemoglobin A1c (HbA1c) and lower fasting and postprandial glucose concentrations. The glucose-dependent mechanism of sitagliptin is distinct from the mechanism of sulfonylureas, which increase insulin secretion even when glucose levels are low and can lead to hypoglycemia in patients with type 2 diabetes and in normal subjects. Sitagliptin is a potent and highly selective inhibitor of the enzyme DPP-4 and does not inhibit the closely-related enzymes DPP-8 or DPP-9 at therapeutic concentrations.

Pharmacodynamic effects

General

In patients with type 2 diabetes, administration of single oral doses of Sitagliptin leads to inhibition of DPP-4 enzyme activity for a 24-hour period, resulting in a 2- to 3-fold increase in circulating levels of active GLP-1 and GIP, increased plasma levels of insulin and C-peptide, decreased glucagon concentrations, reduced fasting glucose, and reduced glucose excursion following an oral glucose load or a meal.

Sitagliptin did not lower blood glucose or cause hypoglycemia, suggesting that the insulinotropic and glucagon suppressive actions of the drug are glucose dependent.

Effects on blood pressure

Hypertensive patients on one or more anti-hypertensive drugs (including angiotensin-converting enzyme inhibitors, angiotensin-II antagonists, calcium-channel blockers, beta-blockers and diuretics), co-administration with Sitagliptin was generally well tolerated. In these patients, Sitagliptin had a modest blood pressure lowering effect; 100 mg per day of Sitagliptin reduced 24-hour mean ambulatory systolic blood pressure by approximately 2 mm Hg. Reductions have not been observed in subjects with normal blood pressure.

Cardiac Electrophysiology

At the recommended dose of 100 mg, there was no effect on the QTc interval obtained at the peak plasma concentration, or at any other time during the study. Following the 800-mg dose, the maximum increase in the placebo-corrected mean change in QTc from baseline at 3 hours post dose was 8.0 msec. This small increase was not considered to be clinically significant. At the 800-mg dose, peak sitagliptin plasma concentrations were approximately 11 times higher than the peak concentrations following a 100-mg dose.

Pharmacokinetics Properties:

The pharmacokinetics of sitagliptin have been extensively characterized in healthy subjects and patients with type 2 diabetes. After oral administration of a 100-mg dose to healthy subjects, sitagliptin was rapidly absorbed, with peak plasma concentrations (median T_{max}) occurring 1 to 4 hours post-dose. Plasma AUC of sitagliptin increased in a dose-proportional manner. The pharmacokinetics of sitagliptin were generally similar in healthy subjects and in patients with type 2 diabetes.

Absorption

The absolute bioavailability of sitagliptin is approximately 87%. Since co-administration of a high-fat meal with Sitagliptin had no effect on the pharmacokinetics, Sitagliptin may be administered with or without food.

Distribution

The mean volume of distribution at steady state following a single 100-mg intravenous dose of sitagliptin to healthy subjects is approximately 198 liters. The fraction of sitagliptin reversibly bound to plasma proteins is low (38%).

Biotransformation

Sitagliptin is primarily eliminated unchanged in urine, and metabolism is a minor pathway. Approximately 79% of sitagliptin is excreted unchanged in the urine.

Following a [14C] sitagliptin oral dose, approximately 16% of the radioactivity was excreted as metabolites of sitagliptin. Six metabolites were detected at trace levels and are not expected to contribute to the plasma DPP-4 inhibitory activity of sitagliptin. In vitro studies indicated that the primary enzyme responsible for the limited metabolism of sitagliptin was CYP3A4, with contribution from CYP2C8.

Elimination

Elimination of sitagliptin occurs primarily via renal excretion and involves active tubular secretion. Sitagliptin is a substrate for human organic anion transporter-3 (hOAT-3), which may be involved in the renal elimination of sitagliptin. The clinical relevance of hOAT-3 in sitagliptin transport has not been established. Sitagliptin is also a substrate of p-glycoprotein, which may also be involved in mediating the renal elimination of sitagliptin. However, cyclosporine, a p-glycoprotein inhibitor, did not reduce the renal clearance of sitagliptin.

Characteristics in Patients

Renal Impairment:

To achieve plasma concentrations of sitagliptin similar to those in patients with normal renal function, lower dosages are recommended in patients with eGFR <45 mL/min/1.73 m².

Hepatic Impairment:

No dosage adjustment for Sitagliptin is necessary for patients with mild or moderate hepatic impairment.

There is no clinical experience in patients with severe hepatic impairment (Child-Pugh score >9). However, because sitagliptin is primarily renally eliminated, severe hepatic impairment is not expected to affect the pharmacokinetics of sitagliptin.

Elderly:

No dosage adjustment is required based on age. Age did not have a clinically meaningful impact on the pharmacokinetics of sitagliptin based on a population pharmacokinetic analysis. Elderly subjects (65 to 80 years) had approximately 19% higher plasma concentrations of sitagliptin compared to younger subjects.

Pediatric:

No studies with Sitagliptin have been performed in pediatric patients.

Gender:

No dosage adjustment is necessary based on gender. Gender had no clinically meaningful effect on the pharmacokinetics of sitagliptin.

Race:

No dosage adjustment is necessary based on race. Race had no clinically meaningful effect on the pharmacokinetics of sitagliptin including patients of white, Hispanic, black, Asian, and other racial groups.

Body Mass Index (BMI):

No dosage adjustment is necessary based on BMI. Body mass index had no clinically meaningful effect on the pharmacokinetics of sitagliptin.

Type 2 Diabetes:

The pharmacokinetics of sitagliptin in patients with type 2 diabetes are generally similar to those in healthy subjects.

5. INDICATION:

Monotherapy

Sitagliptin is indicated as an adjunct to diet and exercise to improve glycemic control in patients with type 2 diabetes mellitus.

Combination with metformin

Sitagliptin is indicated in patients with type 2 diabetes mellitus to improve glycemic control in combination with metformin as initial therapy or when the single agent alone, with diet and exercise, does not provide adequate glycemic control. Initial combination therapy or maintenance of combination therapy may not be appropriate for all patients. These management options are left to the discretion of the health care provider.

Combination with a sulphonylurea

Sitagliptin is indicated in patients with type 2 diabetes mellitus to improve glycemic control in combination with a sulphonylurea when treatment with maximal tolerated dose of sulphonylurea alone, with diet and exercise, does not provide adequate glycemic control and when metformin is inappropriate due to contraindications or intolerance.

Combination with metformin and a sulphonylurea

Sitagliptin is indicated in patients with type 2 diabetes mellitus to improve glycemic control in combination with metformin and a sulphonylurea when dual therapy with these two agents and with diet and exercise does not provide adequate glycemic control.

Combination with a peroxisome proliferator-activated receptor gamma (PPAR γ) agonist:

Sitagliptin is indicated in patients with type 2 diabetes mellitus to improve glycemic control in combination with a PPAR γ agonist (i.e. thiazolidinediones) when diet and exercise, plus the single agent do not provide adequate glycemic control.

Combination with metformin and a PPAR γ agonist

Sitagliptin is indicated in patients with type 2 diabetes mellitus to improve glycemic control in combination with metformin and a PPAR γ agonist (i.e., thiazolidinediones) when dual therapy with these agents, with diet and exercise, does not provide adequate glycemic control.

Combination with Insulin

Sitagliptin is indicated in patients with type 2 diabetes mellitus as an adjunct to diet and exercise to improve glycemic control in combination with insulin (with or without metformin).

6. POSOLOGY AND METHOD OF ADMINISTRATION:

The recommended dose of Sitagliptin is 100 mg once daily as monotherapy or as combination therapy with metformin, a sulfonylurea, insulin (with or without metformin), a PPAR γ agonist (i.e., thiazolidinediones), metformin plus a sulfonylurea, or metformin plus a PPAR γ agonist. Sitagliptin can be taken with or without food.

When Sitagliptin is used in combination with a sulfonylurea or with insulin, a lower dose of sulfonylurea or insulin may be considered to reduce the risk of sulfonylurea- or insulin-induced hypoglycaemia.

Patients with Renal Impairment

Because there is a dosage adjustment based upon renal function, assessment of renal function is recommended prior to initiation of Sitagliptin and periodically thereafter.

For patients with mild renal impairment (estimated glomerular filtration rate [eGFR] ≥ 60 mL/min/1.73 m² to < 90 mL/min/1.73 m²), no dosage adjustment for Sitagliptin is required.

For patients with moderate renal impairment (eGFR ≥ 45 mL/min/1.73 m² to < 60 mL/min/1.73 m²), no dosage adjustment for Sitagliptin is required.

For patients with moderate renal impairment (eGFR ≥ 30 mL/min/1.73 m² to 45 mL/min/1.73 m²), the dose of Sitagliptin is 50 mg once daily.

For patients with severe renal impairment (eGFR ≥ 15 mL/min/1.73 m² to < 30 mL/min/1.73 m²) or with end-stage renal disease (ESRD) (eGFR < 15 mL/min/1.73 m²), including those requiring hemodialysis or peritoneal dialysis, the dose of Sitagliptin is 25 mg once daily. Sitagliptin may be administered without regard to the timing of dialysis.

Pediatric population

Sitagliptin should not be used in children and adolescents 10 to 17 years of age because of insufficient efficacy. Sitagliptin has not been studied in pediatric patients under 10 years of age.

7. METHOD OF ADMINISTRATION

Sitagliptin can be taken with or without food.

8 CONTRAINDICATION:

Sitagliptin is contraindicated in patients who are hypersensitive to any components of this product.

9. SPECIAL WARNINGS AND PRECAUTIONS FOR USE

General

Sitagliptin should not be used in patients with type 1 diabetes or for the treatment of diabetic ketoacidosis.

Pancreatitis:

There have been reports of acute pancreatitis, including fatal and non-fatal hemorrhagic or necrotizing pancreatitis, in patients taking sitagliptin. Patients should be informed of the characteristic symptom of acute pancreatitis: persistent, severe abdominal pain. Resolution of pancreatitis has been observed after discontinuation of sitagliptin. If pancreatitis is suspected, Sitagliptin and other potentially suspect medicinal products should be discontinued.

Use in Patients with Renal Impairment:

Sitagliptin is renally excreted. To achieve plasma concentrations of Sitagliptin similar to those in patients with normal renal function, lower dosages are recommended in patients with eGFR < 45 mL/min/1.73 m²), as well as in ESRD patients requiring hemodialysis or peritoneal dialysis.

Hypoglycemia in Combination with a Sulfonylurea or with Insulin:

As is typical with other antihyperglycemic agents, hypoglycemia has been observed when Sitagliptin was used in combination with insulin or a sulfonylurea. Therefore, to reduce the risk of sulfonylurea- or insulin-induced hypoglycemia, a lower dose of sulfonylurea or insulin may be considered.

Hypersensitivity Reactions:

These reactions include anaphylaxis, angioedema, and exfoliative skin conditions including Stevens-Johnson syndrome. Because these reactions are reported voluntarily from a population of uncertain size, it is generally not possible to reliably estimate their frequency or establish a causal relationship to drug exposure. Onset of these reactions occurred within the first 3 months after initiation of treatment with Sitagliptin, with some reports occurring after the first dose. If a hypersensitivity reaction is suspected, discontinue Sitagliptin, assess for other potential causes for the event, and institute alternative treatment for diabetes.

Severe and Disabling Arthralgia:

There have been severe and disabling arthralgia in patients taking DPP-4 inhibitors. The time to onset of symptoms following initiation of drug therapy varied from one day to years. Patients experienced relief of symptoms upon discontinuation of the medication. A subset of patients experienced a recurrence of symptoms when restarting the same drug or a different DPP-4 inhibitor. Consider DPP-4 inhibitors as a possible cause for severe joint pain and discontinue drug if appropriate.

Bullous Pemphigoid:

Bullous pemphigoid requiring hospitalization have been reported with DPP-4 inhibitor use. In reported cases, patients typically recovered with topical or systemic immunosuppressive treatment and discontinuation of the DPP-4 inhibitor. Tell patients to report development of blisters or erosions while receiving Sitagliptin. If bullous pemphigoid is suspected, Sitagliptin should be discontinued and referral to a dermatologist should be considered for diagnosis and appropriate treatment.

10. INTERACTION WITH OTHER MEDICINAL PRODUCTS AND OTHER FORMS OF INTERACTION

Sitagliptin did not have clinically meaningful effects on the pharmacokinetics of the following: metformin, rosiglitazone, glyburide, simvastatin, warfarin, and oral contraceptives. Based on these data, sitagliptin does not inhibit CYP isozymes CYP3A4, 2C8, or 2C9. Based on in vitro data, sitagliptin is also not expected to inhibit CYP2D6, 1A2, 2C19 or 2B6 or to induce CYP3A4.

Co-administration of multiple twice-daily doses of metformin with sitagliptin did not meaningfully alter the pharmacokinetics of sitagliptin in patients with type 2 diabetes.

Concomitant medications did not have a clinically meaningful effect on the pharmacokinetics of sitagliptin. Medications assessed were those that are commonly administered to patients with type 2 diabetes including cholesterol-lowering agents (e.g., statins, fibrates, ezetimibe), anti-platelet agents (e.g., clopidogrel), antihypertensives (e.g., ACE inhibitors, angiotensin receptor blockers, beta-blockers, calcium channel blockers, hydrochlorothiazide), analgesics and non-steroidal anti-inflammatory agents (e.g., naproxen, diclofenac, celecoxib), anti-depressants (e.g., bupropion, fluoxetine, sertraline), antihistamines (e.g., cetirizine), proton-pump inhibitors (e.g., omeprazole, lansoprazole), and medications for erectile dysfunction (e.g., sildenafil).

Patients receiving digoxin should be monitored appropriately.

No dosage adjustment of digoxin or Sitagliptin is recommended.

No dosage adjustment for Sitagliptin is recommended when co-administered with cyclosporine or other p-glycoprotein inhibitors (e.g., ketoconazole).

11. PREGNANCY AND NURSING MOTHERS:

Pregnancy

There are no adequate and well-controlled studies in pregnant women; therefore, the safety of Sitagliptin in pregnant women is not known. Sitagliptin like other oral anti-hyperglycemic agents, is not recommended for use in pregnancy.

Nursing Mothers

Sitagliptin is secreted in the milk of lactating rats. It is not known whether sitagliptin is secreted in human milk. Therefore, Sitagliptin should not be used by a woman who is nursing.

12. UNDESIRABLE EFFECTS:

Sitagliptin was generally well tolerated in controlled clinical studies as both monotherapy and combination therapy, with discontinuation of therapy due to clinical adverse experiences similar to placebo.

In four placebo-controlled clinical studies, as both monotherapy (one study of 18- and one of 24-week duration) and add-on combination therapy with metformin or pioglitazone (both of 24-week duration), there were 1082 patients treated with Sitagliptin 100 mg once daily and 778 patients given placebo. (Two of these studies also included 456 patients treated with Sitagliptin 200 mg daily, two times the recommended daily dose.)

There were no drug-related adverse reactions reported that occurred with an incidence of $\geq 1\%$ in patients receiving Sitagliptin 100 mg. Overall, the safety profile of the 200-mg daily dose was similar to that of the 100-mg daily dose.

In a prespecified pooled analysis of the above studies, the overall incidence of adverse experiences of hypoglycemia in patients treated with Sitagliptin 100 mg was similar to placebo (1.2% vs. 0.9%). The incidences of selected gastrointestinal adverse experiences in patients treated with Sitagliptin or placebo were: abdominal pain (Sitagliptin, 2.3%; placebo, 2.1%), nausea (1.4%, 0.6%), vomiting (0.8%, 0.9%), and diarrhea (3.0%, 2.3%).

Adverse reactions of hypoglycemia were based on all reports of symptomatic hypoglycemia; a concurrent glucose measurement was not required.

Add-on Combination with a Sulfonylurea: In a 24-week placebo-controlled study of SITAGLIPTIN 100 mg in combination with glimepiride or with glimepiride and metformin (SITAGLIPTIN, N=222; placebo, N=219), the drug-related adverse reaction reported in $\geq 1\%$ of patients treated with SITAGLIPTIN and more commonly than in patients treated with placebo was hypoglycaemia (SITAGLIPTIN, 9.5%; placebo, 0.9%).

Add-on Combination with Metformin and a PPAR γ Agonist: In a placebo-controlled study of SITAGLIPTIN 100 mg in combination with metformin and rosiglitazone (SITAGLIPTIN, N=170; placebo, N=92), the drug-related adverse reactions reported through the primary time point at Week 18 in $\geq 1\%$ of patients treated with SITAGLIPTIN and more commonly than in patients treated with placebo were: headache (SITAGLIPTIN, 2.4%; placebo, 0.0%), diarrhea (1.8%, 1.1%), nausea (1.2%, 1.1%), hypoglycemia (1.2%, 0.0%), and vomiting (1.2%, 0.0%). Through Week 54, the drug-related adverse reactions reported in $\geq 1\%$ of patients treated with SITAGLIPTIN and more commonly than in patients treated with placebo were: headache (2.4%, 0.0%), hypoglycaemia (2.4%, 0.0%), upper respiratory tract infection (1.8%, 0.0%), nausea (1.2%, 1.1%), cough (1.2%, 0.0%), fungal skin infection (1.2%, 0.0%), peripheral edema (1.2%, 0.0%), and vomiting (1.2%, 0.0%).

Initial Combination Therapy with Metformin: In a 24-week placebo-controlled factorial study of initial therapy with sitagliptin 100 mg in combination with metformin at 1000 mg or 2000 mg per day (administered as sitagliptin 50 mg/metformin 500 mg or 1000 mg twice daily), the drug-related adverse reactions reported in $\geq 1\%$ of patients treated with sitagliptin plus metformin (N=372) and more commonly than in patients treated with metformin alone (N=364) were: diarrhea (sitagliptin plus metformin, 3.5%; metformin, 3.3%), dyspepsia (1.3%; 1.1%), flatulence (1.3%; 0.5%), vomiting (1.1%; 0.3%), and headache (1.3%; 1.1%). The incidence of hypoglycemia was 1.1% in patients given sitagliptin in combination with metformin and 0.5% in patients given metformin alone.

Initial Combination Therapy with a PPAR γ Agonist:

The incidence of (symptomatic) hypoglycemia was in patients given Sitagliptin in combination with pioglitazone and in patients given pioglitazone.

Add-on Combination with Insulin:

Patients receiving Sitagliptin as add-on therapy while undergoing insulin intensification (with or without metformin), there were no drug-related adverse reactions reported that occurred with an incidence of $\geq 1\%$ in patients treated with Sitagliptin 100 mg and more commonly than in patients treated with placebo.

Pancreatitis:

No clinically meaningful changes in vital signs or in ECG (including in QTc interval) were observed in patients treated with Sitagliptin.

TECOS Cardiovascular Safety Study:

The adverse event profile in patient's ≥ 75 years of age was generally similar to the overall population.

In the intention-to-treat population, among patients who were using insulin and/or a sulfonylurea at baseline, the incidence of severe hypoglycemia was in Sitagliptin -treated patients; among patients who were not using insulin and/or a sulfonylurea at baseline, the incidence of severe hypoglycemia was in Sitagliptin -treated patients. The incidence of adjudication-confirmed pancreatitis events was in Sitagliptin -treated patients. The incidence of adjudication-confirmed malignancy events was 3.7% in Sitagliptin -treated patients.

Pediatric population:

In a clinical study with SITAGLIPTIN 100 mg in pediatric patients aged 10 to 17 years with type 2 diabetes, there were no drug-related adverse reactions reported through the 54-week treatment period in more than 1 patient in the SITAGLIPTIN group (N=95) and more commonly than in patients in the placebo group (N=90).

There were no clinically relevant differences between the SITAGLIPTIN and placebo groups through Week 54 in laboratory safety endpoints, vital signs, indices of adiposity, or growth and development endpoints.

Post marketing Experience:

Additional adverse reactions have been identified during post marketing use of Sitagliptin as monotherapy and/or in combination with other antihyperglycemic agents. Because these reactions are reported voluntarily from a population of uncertain size, it is generally not possible to reliably estimate their frequency or establish a causal relationship to drug exposure.

Hypersensitivity reactions including anaphylaxis, angioedema, rash, urticaria; cutaneous vasculitis, and exfoliative skin conditions including Stevens-Johnson syndrome; acute pancreatitis, including fatal and non-fatal hemorrhagic and necrotizing pancreatitis; worsening renal function, including acute renal failure (sometimes requiring dialysis); bullous pemphigoid; severe and disabling arthralgia; upper respiratory tract infection; nasopharyngitis; constipation; vomiting; headache; myalgia; pain in extremity; back pain; pruritus; thrombocytopenia.

13. SIGNS & SYMPTOMS OF OVERDOSE AND TREATMENT:

Single doses of up to 800 mg Sitagliptin were generally well tolerated. Minimal increases in QTc, not considered to be clinically relevant. There is no experience with doses above 800 mg. There were no dose-related clinical adverse reactions observed with Sitagliptin with doses of up to 600 mg per day for periods of up to 10 days and 400 mg per day for periods of up to 28 days.

In the event of an overdose, it is reasonable to employ the usual supportive measures, e.g., remove unabsorbed material from the gastrointestinal tract, employ clinical monitoring (including obtaining an electrocardiogram), and institute supportive therapy if required.

Sitagliptin is modestly dialyzable. In clinical studies, approximately 13.5% of the dose was removed over a 3- to 4-hour hemodialysis session. Prolonged hemodialysis may be considered if clinically appropriate. It is not known if sitagliptin is dialyzable by peritoneal dialysis.

14. Effects on Ability to Drive and Use Machine

There is no information to suggest that SITAGLIPTIN affects your ability to drive a car or operate machinery.

15. Instructions for Use

SITACORD can be taken with or without food.

16. LIST OF EXCIPIENTS:

Core tablet

Microcrystalline Cellulose PH 200 LM
Dicalcium Phosphate Anhydrous Di- Cafos A 150
Croscarmellose Sodium (ACDISOL)
Magnesium stearate
Sodium Stearyl Fumarate

Film-coating

Polyvinyl Alcohol-Part. Hydrolyzed (E1203)
Titanium dioxide (E171)
Macrogol / PEG (E1521)
Talc (E553b)
Iron oxide yellow (E172)
Iron Oxide Red (E172)

17. STORAGE CONDITIONS:

Store below 30 °C.
Keep out of the reach and sight of children.

18. DOSAGE FORMS AND PACKAGING AVAILABLE:

Sitagliptin Tablets 50 mg and 100 mg are packed in Desiflex Alu. blisters of 10 Tablets. 3 such blisters are packed in one printed carton.

19. SHELF LIFE:

24 months.

20. NAME AND ADDRESS OF MANUFACTURER / MARKETING AUTHORIZATION HOLDER:

Intas Pharmaceuticals Limited.
Plot No 5-14, Pharmed,
Near Village Matoda,
Sarkhej-Bavla National Highway No. 8-A,

Sanand Taluka
Ahmedabad, Gujarat
IN 382213, INDIA

Product Registration Holder

Accord Healthcare Sdn. Bhd.
Suite 12A-15, Level 12A, Wisma Zelan
No 1, Jalan Tasik Permaisuri 2, Bandar Tun Razak,
56000 Kuala Lumpur.

21. DATE OF REVISION:

04/05/2024

(Suggested font size: 7, as measure in Times New Roman)